CLINICAL TRIAL FAILURES · DISCONTINUATION RATE · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

BCL-2: one molecule stopped early across 5 trials

5 stopped trials, all of one molecule, Venetoclax (a small molecule). 3 of the stops were the trial's own verdict and 1 followed a decision taken elsewhere; 1 cannot be established. Against 5.2% for all oncology trials with a resolved drug target and 4.5% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

4.8%
5 of 104 closed trials (95% CI 2.1%–10.8%)

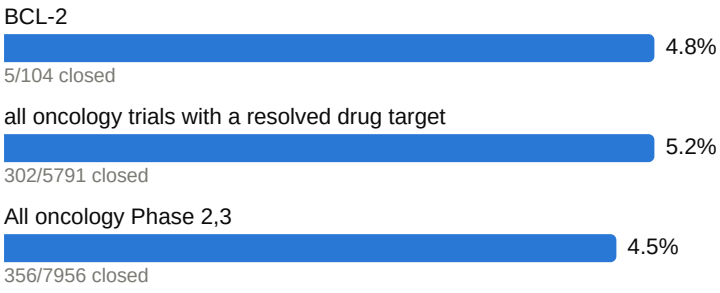
4 / 0 / 1
efficacy / safety / both (adds to 5)

30.3%
of 343 trials have closed · 239 still open or unresolved

0.0%
stopped for a biological reason within 3 years of starting (95% CI 0.0%–0.0%; 186 still at risk; 1.5% area-wide)

1 of 4 **Completed and missed the primary endpoint.** Of 60 completed trials in this segment, 4 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 1 missed and 3 met. Counted separately and never in the rate above; a floor, not a rate. 24 of the 55 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 182 distinct experimental drugs across 343 trials (small molecule 338, antibody 90, unknown 47, protein 16). Most tested: VENETOCLAX (305), AZACITIDINE (88), OBINUTUZUMAB (37), CYTARABINE (37), IBRUTINIB (31). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 5 stops came from 5 sponsor-asset programmes across 5 sponsors; the largest (Roche) contributed 1; removing that programme's trials from both sides leaves 3.9%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 9 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 13.5%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	1	23	4.3%	0.8%–21.0%
2018–2020	4	53	7.5%	3.0%–17.9%
2021–2024	0	28	0.0%	0.0%–12.1%

Where the stops sit

Efficacy / futility: 4, Efficacy and safety: 1.
Sponsors with most stops: Novartis (1), Roche (1), Ono Pharmaceutical Co., Ltd. (1), M.D. Anderson Cancer Center (1), Fondazione Italiana Linfomi - ETS (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04150029	2	Novartis NVS	SABATOLIMAB, VENETOCLAX, AZACITIDINE	2024-10	Efficacy	Novartis made a decision to early terminate the sabatolimab clinical development program, including the CMBG453C12201 trial, following the negative results from other 2 trials.

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04029688	2	Roche	IDASANUTLIN, VENETOCLAX, CYCLOPHOSPHAMIDE ANHYDROUS, TOPOTECAN	2024-05	Efficacy + safety	Study was terminated by the sponsor based on insufficient tolerability and efficacy to proceed beyond Part 1b.
NCT03176277	2	Ono Pharmaceutical Co., Ltd.	TAMNORZATINIB, VENETOCLAX	2022-12	Efficacy	Protocol defined futility criteria
NCT03634228	2	M.D. Anderson Cancer Center	CYTARABINE, MILADEMETAN, VENETOCLAX	2022-04	Efficacy	This study was terminated early due to lack of adequate response, and did not move to the Phase II portion of the study
NCT03552692	2	Fondazione Italiana Linfomi - ETS	VENETOCLAX	2020-03	Efficacy	efficacy lower than expected

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 30.3% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta: enter your work email at **clinicaltrialfailures.com/asset-check** and it opens straight away.

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.